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Health and Human Services Agency
DEPARTMENT OF MANAGED HEALTH CARE
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March 20, 2026

VIA ELECTRONIC MAIL

Karen Choate
Blue Cross of California
1121 L Street, Suite 500
Sacramento, CA 95814

Re: Proposed Contract Termination with Valley Children's Hospital
Block Transfer Filing No. 2026-HOSP-294
Filed on March 18, 2026

Dear Ms. Choate,

The Department of Managed Health Care (Department) is reviewing the information in the above-referenced Block Transfer Filing (Filing) submitted by **Blue Cross of California** (Plan) for compliance with the requirements of the Knox-Keene Health Care Service Plan Act of 1975, as amended (Cal. Health & Safety Code § 1340 *et seq.*) and the regulations promulgated thereunder (Cal. Code Regs., Tit. 28, § 1000 *et seq.*).

In the event of contract termination, the Filing proposes to remove Valley Children's Hospital (Terminating Hospital) from the Plan's hospital network. The effective date of contract termination between the Terminating Hospital and the Plan (Termination Date) is currently **July 1, 2026**.

Please review the following comments and feel free to contact me if you would like to discuss any of the issues before filing the Plan's response.

1. Please submit an Enrollee Transfer Notice for the Department's review.

Plan Response1: The Plan inadvertently provided the provider term notice instead of the Enrollee Notice. Please refer to Enrollee Notice below:



Valley%20Children's
%20Post%20Term%2

2. Please confirm the Plan will utilize the most recently approved eFiling Enrollee Transfer Notice Templates for this Block Transfer Filing.

Plan Response2: The Plan will utilize the most recently approved eFiling Enrollee Transfer Notice Templates for this Block Transfer Filing.

3. Please confirm that the Plan is aware that it cannot transition its enrollees or send Enrollee Transfer Notices without the Department's approval.

Plan Response3: The Plan will not transition its enrollees or send Enrollee Transfer Notices without the Department's approval.

4. Please confirm whether the hospital termination is part of a health system termination. If yes, please confirm the health system and whether the contract is a stand-alone contract.

Plan Response4: The hospital termination is not part of a health system termination.

5. Please confirm whether the contract termination between the Plan and Valley Children's Hospital impacts Point of Service (POS) enrollees.

Plan Response5: The contract termination between the Plan and the Terminating Provider impacts Point of Service (POS) enrollees.

6. The Plan indicates via the Valley Children's_NON-ER Bed Days and Admits document there were 27 HMO bed days and 5 HMO admits; however, the Plan states there are 0 HMO enrollees impacted by the possible termination. Please clarify the discrepancy.

Plan Response6: No enrollees were impacted as there were no provider groups that primarily admit to the Terminating Hospital as the denoted by the grid below:

VALLEY CHILDREN'S HOSPITAL		
HMO Medical Group	Prior Year Non-ER Bed Days	Prior Year Non-ER Admits
Allcare IPA	9	2
Sante Physicians IPA	11	1
Key Medical Group	7	2
HMO Sub-Totals	27	5

Note: Medical groups on record that primarily admit to the terminating hospital: <i>NONE</i>		
PPO Enrollees	Prior Year Non-ER Bed Days	Prior Year Non-ER Admits
	400	45
Grand total for HMO & PPO	427	50

7. Please confirm whether the PPO providers that routinely utilize the Terminating Hospital have adequate admitting privileges for PCPs and specialists to at least one of the identified Alternate Hospitals containing all services that will be lost should the contract ultimately terminate.

Plan Response7: PPO participating providers (PCPs and specialists) are not assigned to a specific hospital and may admit members to any in-network hospital consistent with the member's benefits and the hospital's admitting processes. Therefore, if the Valley Children's Hospital contract terminates, PPO providers will continue to have inpatient admitting access for members through the Plan's network at one or more in-network Alternate Hospitals. These facilities collectively provide inpatient hospital services, and pediatric tertiary services are available at the identified children's hospitals (Lucile Packard, CHLA, and UCSF Benioff Oakland), which are capable of furnishing the specialized pediatric services that would be lost with the termination of Valley Children's.

8. The Plan indicates that "Tertiary Care Hospital "is available at the Terminating Hospital.
- Please list out all services that fall under tertiary care
 - Please confirm which Alternate Hospital(s) provides each service(s).

Plan Response8: Please refer to grid below:

Valley Children's publicly confirmed pediatric tertiary services	Alternate Hospitals offering services (publicly confirmed)
Cancer & Blood Disorders / Hematology-Oncology	CHLA; LPCH; UCSF Benioff Children's Hospital Oakland (aka Children's Hospital & Research Ctr-Oakland)
Gastroenterology	CHLA; LPCH; UCSF Benioff Children's Hospital Oakland
Maternal Fetal Center / Maternal Fetal Medicine	CHLA; LPCH; UCSF Benioff Children's Hospital Oakland
Neonatology / NICU	CHLA; LPCH; UCSF Benioff Children's Hospital Oakland

Valley Children's publicly confirmed pediatric tertiary services	Alternate Hospitals offering services (publicly confirmed)
ECMO (NICU specialties/services)	CHLA; LPCH; UCSF Benioff Children's Hospital Oakland
Neuroscience Center / Neurology	CHLA; LPCH; UCSF Benioff Children's Hospital Oakland
Orthopedics	CHLA; LPCH; UCSF Benioff Children's Hospital Oakland
Pediatric Surgery	CHLA; LPCH; UCSF Benioff Children's Hospital Oakland
Heart Center / Cardiology	CHLA; LPCH; UCSF Benioff Children's Hospital Oakland
Trauma Services (Level II)	CHLA; LPCH; UCSF Benioff Children's Hospital Oakland-all Level I)

9. The Plan indicates that the services below are available at the Terminating Hospital. As the services are available only at Alternate Hospitals that are outside of geographical access, please confirm how enrollees will maintain these services post termination.
- Acute Neonatal
 - Acute Pediatric
 - "Tertiary Care Hospital" services
 - Trauma Center - Level II

Plan Response9: Publicly available information indicates that Saint Agnes Medical Center and Clovis Community Medical Center offer NICU (acute neonatal) services and describe these services as provided in partnership with the Terminating Hospital. Accordingly, if the termination occurs, it may affect the availability of certain specialized neonatal/pediatric services locally, depending on the clinical staffing and referral/transfer arrangements used at these facilities. Children's LA, Children's Oakland, & Lucile Packard Children's are the closest alternates to the Terminating Hospital that provides specialized pediatric care. In California, there are only approximately a dozen children-only hospitals that provide specialized pediatric care. As Plan members are not assigned to hospitals, they can access any Plan in-network hospital in California for specialized care, if needed.

Additionally, if a covered service is unavailable, inaccessible, or cannot be provided promptly at a network hospital, the Plan allows members to receive the service at a non-participating hospital and reimbursed at in-network benefit levels. As mentioned previously, Plan enrollees are not restricted to specific hospitals and may be admitted to any participating Plan hospital, including these,

irrespective of the distance from the Terminating Hospital. Please refer to Plan Response8 regarding Tertiary and Trauma Services.

10. The Plan indicates that Cancer services are available at the Terminating Hospital. Please specify which Cancer services are available at the Terminating Hospital and confirm which Alternate Hospital(s) provides each service(s).

Plan Response10: The OSPD/HCAI annual disclosure dataset shows a positive indicator for Oncology for the Terminating Hospital as well as all the listed alternate hospitals. The Plan relies on that standardized public indicator to confirm that each facility reports offering oncology-related services. Because "Oncology" is a broad service-line label and hospital websites are not standardized or comprehensive, the Plan used the OSHPD/HCAI indicator as the consistent basis for determining that at least one of the alternate hospitals (i.e., Clovis, Lucile Packard, St. Agnes) have the same service category available.

11. The Plan's PPO line of business is licensed by the Department. Please confirm the following:

- a. The Plan will send an Enrollee Transfer Notice approved by the Department to all PPO enrollees with a 15 mile radius of the Terminating Hospital.

Plan Response11: The Plan will send Department-approved Enrollee Transfer Notices to all PPO enrollees residing within a 15-mile radius of the Terminating Hospital.

12. The Department notes that the Plan has chosen Directly Contracted Network (DCN) as a group that admits to the Terminating Hospital, for the Enrollee Access section of the Block Transfer Filing Form. The Department also notes that the Plan has stated there are no impacted medical groups. Please state whether DCN is impacted by the pending termination or if DCN was selected because a provider group must be chosen for Enrollee Access section of the Block Transfer Filing Form.

Plan Response12: The Plan confirms there are no impacted medical groups. The Plan also confirms DCN is NOT impacted by the pending termination and was only selected because the portal requires the selection of a provider group for the Enrollee Access section of the Block Transfer Filing Form.

13. For each Alternate Hospital, please summarize or confirm the following:

- a. The Plan contacted the hospital regarding the Block Transfer,
b. Any concerns expressed by the hospital regarding its ability to accommodate the hospital-based needs of enrollees redirected by the Block Transfer,

- c. Whether the Plan has sent or received a notice of contract termination from the Alternate Hospital, and if so, whether that contract is set to terminate within the next six months.

Plan Response13: The Plan outreaches to the hospitals with occupancy rates at 80% or higher and documented any concerns and capacity response(s) in the filing portal. For hospitals with occupancy rates <80%, justification is not provided as to the sufficiency of the Alternate Hospital's capacity to serve additional plan enrollees per §1300.67.1.3(2)(L): "Based upon the data made public on the Office of Statewide Health Planning and Development's website, for each of the proposed Alternate Hospitals, provide the available bed occupancy rate for the most recently completed calendar year. If the rate is at 80% or higher, please provide justification as to the sufficiency of the Alternate Hospital's capacity to serve additional plan enrollees." All alternate hospitals are <80% capacity. The Plan has NOT sent or received notice of contract termination from the Alternate Hospitals.

14. Please confirm whether the HMO providers that routinely utilize the Terminating Hospital have adequate admitting privileges for PCPs and specialists to at least one of the identified Alternate Hospitals containing all services that will be lost should the contract ultimately terminate.

If any services are not available at the Alternate Hospitals, please explain where affected enrollees will receive these services following contract termination.

Plan Response 14: HMO providers that routinely utilize the Terminating Hospital have adequate admitting privileges for PCPs and specialists to at least one of the identified Alternate Hospitals containing all services. If a covered service is unavailable, inaccessible, or cannot be provided promptly at a network hospital, the Plan allows members to receive the service at a non-participating hospital and reimbursed at in-network benefit levels.

15. Please confirm the medical groups that do not utilize the Terminating Hospital as a primary hospital but have admitted enrollees for the most recently completed calendar year, are able to refer their enrollees to other hospitals within the Plan's network for services comparable to those provided by the Terminating Hospital.

Plan Response15: The medical groups that do not utilize the Terminating Hospital as a primary hospital, but have admitted enrollees for the most recently completed calendar year, are able to refer their enrollees to other hospitals within the Plan's network for services comparable to those provided by the Terminating Hospital.

16. In addition to the in-patient services comparison on the Block Transfer portal, please indicate any of the following services that are available at the Terminating Hospital, but not available at the Alternate Hospitals:

a. Any out-patient or in-patient services

Plan Response16: The Plan does not anticipate any problems with the ability of the alternate hospitals to provide the same out-patient and/or in-patient services available at the terminating facility. If there is a service identified that is a covered benefit and only available at the terminating hospital the Plan will make every effort to ensure services are provided at the terminating hospital. When a covered service is not available or accessible at a participating network hospital or specialty facility in the Plan's' network members will be allowed to receive that service at a non-participating hospital as appropriate; reimbursement at in-network benefit level.

17. Please identify the number of enrollees who have a procedure scheduled and authorized by the Plan at the Terminating Hospital post-termination.

Plan Response17: To date, there are no enrollees who have a procedure scheduled and authorized by the Plan at the Terminating Hospital post-termination.

- ~~18. Please confirm that the Plan will provide notice of the contract termination to the following:~~

Plan Comment: The Plan will disregard question 18 per portal conversation with Reviewer-Elizabeth D. on 3/27.

Please submit the Plan's response by **March 27, 2026**, via the Block Transfer portal as a "Response to Comment Letter."

Please contact me if there are any questions regarding the above.

Sincerely,

Elizabeth Doyle

Elizabeth Doyle
Analyst II
Office of Plan Monitoring